

المجلس الطبي المتخصص

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Name: هبه اسماعيل عبد الجواد اسماعيل National ID: 28703270104246	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقير الشخصية

هبة
اسماعيل عبد الجواد اسماعيل
٩ ش مصطفى عصر - بيجام
شبرا الخيمة اول - القليوبية

١٩٨٧/٠٨/٢٧ ٢٨٧٠٣٢٧٠١٠٤٢٤٦

KD4501180

Dms
مصر

٢٠٢٢/٠٨ ٢٨٧٠٣٢٧٠١٠٤٢٤٦
ربة منزل

أنثى
مسلمة
متزوجة
حسام محمد محمد عبد القادر
البطاقة سارية حتى ٢٠٣٠/٠٨/٢٧

١١

MDT FORM

10/11 / 2025 G

Name	د. محمد عبد الرحمن	MRN	٣٠٩٠/2025
Age	37	ID	CAV.٢CV.١٠٤٢٤٦
Menopausal status	premenopausal	Phone	01220311276
NRS SCORE		Marital status	married
BMI		Family history	+ve mother, Aunt
CO morbidity	DM, HTN → not on medications	REASON OF MDT	New admission

Clinical Data

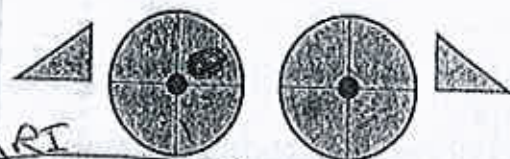
BX:	IDC + Fibroadenomatoid Hyperplasia retroal			PS=1	L
ER:	0/8	PR:	0/8	HER 2:	-ve
Ki67:					

Sonomammography:

Rt breast cancer triple -ve
multi-centric multi focal lesions

Mets Workup:

largest 2.8 cm
CT CAP and bone scan → free



MRI

12-5 o'clock

TNM
3 1 0

Recommendations:

Newly diagnosed since June

Referred for us → for IO + chemo neoadjuvant

For possibility of NACT then Rt MRM

MDT OUTCOME & PLAN

① BCSA test
② Start NACT (AC x 4) + Pembrolizumab 200 mg IV x 8 then
③ For possibility of NACT then Rt MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name: هبة اسماعيل عبدالجواد اسماعيل
RN: 10210125112200
O.B: 27/03/1987
No.: 28703270104246

Location: External Clinic
Sex: Female

Surgical Pathology

Specimen

Specimen Id 2506111206
Accession No SU-25-3767
Sample Type TISSUE

Collection Date 11/06/2025 12:17 PM
Received Date 11/06/2025 12:17 PM

Specimen

- I. Main breast mass.
- II. Medial superior lesion at 1 to 2 o'clock.
- III. Retro areolar deep lesion at 12 o'clock.

GROSS DESCRIPTION

- I. Three tan tissue cores, each measuring 1.8cm in length T.E.
- II. Three dark brownish tissue cores ranging in length from 0.6 to 1.3cm. T.E.
- III. Two tan tissue cores measuring 0.8 and 1.1cm in length and small dark brownish tissue fragment measuring 0.3x0.2x0.1cm. T.E.

MICROSCOPIC

Serial sections examined from specimens labelled (1) and (2) revealed multiple breast tissue cores infiltrated by malignant epithelial neoplasm formed of cords and nests of malignant epithelial cells with poor attempts at tubular formation (score 3). The malignant cells exhibit moderate degree of nuclear atypia (score 2) and infrequent mitotic figure (score 1). The surrounding stroma is sclerotic with focal adenosis.

Tumor infiltrating lymphocytes (low).

No perineural and lymphovascular invasion seen, No insitu component could be detected.

IHC results:

ER: No nuclear staining in tumor cells (Allred score 0/8).

PR: No nuclear staining in tumor cells (Allred score 0/8).

Her2: No membrane reaction in tumor cells (score 0).

Ki67: Proliferation index 30%.

(3) Serial sections examined reveal fragmented breast tissue cores showing proliferation of both stromal and epithelial component. The stroma is loose myxoid. The epithelial component is formed of breast ducts lined by dual layer of inner cuboidal and outer myoepithelial cells. No evidence of malignancy in serials examined.

IMMUNOHISTOCHEMICAL RESULT

DIAGNOSIS

I. Main breast mass, tru cut biopsy:

II. Medial superior lesion at 1 to 2 o'clock, tru cut biopsy:

Both represent invasive duct carcinoma, grade 2 (Triple negative, high proliferation index breast carcinoma).

III. Retro areolar deep lesion at 12 o'clock:

Fibroadenomatoid hyperplasia.

No malignancy.

Signature

Prof. Dr. Wessam Osman

Prof. Dr. Lobna Shash

Dr. Nermeen Mohamed



Patient Name	هبة اسماعيل عبدالجواد اسماعيل	PID	1021012511 2200	D/T	04-06-2025
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SONOMAMMOGRAPHY OF BOTH BREASTS

Procedure: Digital mammography and real time ultrasonography of both breasts were performed.

Findings:

- Mammography shows right multiple radio-dense irregular speculated lesions, two of them seen hyper echoic with speculated margins, with another smaller lesion seen at the pre-pectoral lesion which was not detected in Ultrasound
- Complementary ultrasound confirmed the presence of two hypo-echoic hypo-echoic lesions with irregular margins seen at o'clock in position 1 cm from the nipple
- Another two larger lesions with speculated margins detected at 3 o'clock in position located at 5 and 3 cm from the nipple the largest measuring 1.7x1.6 cm in diameter
- Normal appearance of the subcutaneous and retromammary fat on both sides.
- The breast parenchyma shows scattered fibroglandular densities (ACR breast density b).
- No suspicious solid or cystic masses or clusters of microcalcifications were detected in left breast.
- Right suspicious looking enlarged axillary LN

Impression:

Right multifocal breast malignant looking lesions (BIRADS V) as described for histopathological correlation.

Right suspicious looking axillary LN.

UNREMARKABLE LEFT BREAST STUDY (BIRADS 1).

Much obliged
R/Youssef Nour
Norhan Mamdouh, Msc.



Patient Name: هبة اسماعيل عبدالجواد	Patient ID: 420329
Study Date: 08/10/2025 11:25:00 O	

CT scan of the Chest , abdomen & Pelvis :

Chest :

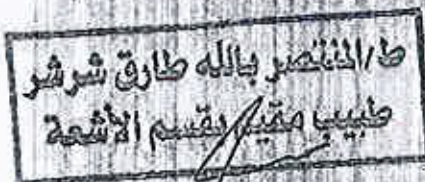
- Right breast irregular shaped mass lesion measuring about (2.5x1.5 cm) with spiculated outline right axillary LN measuring (1.5 cm).
- Clear both lungs with no definite focal lesions, infiltrations or bronchiectatic changes.
- No pleural effusion.
- Normal cardiac size with no pericardial effusion.
- No gross mediastinal masses or lymphadenopathy are detected.

Abdomen and pelvis :-

- Average sized liver showing smooth outlines with no definite focal lesions or dilated IHBR. Patent main portal vein and its central branches.
- Normal CT appearance of the spleen, adrenal glands, pancreas, kidneys, IVC and aorta.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs.

PROF. DR. MOHAMED ABD ELLATEF, MD

NORA





Heba Ismail Abd Al-Gawad

الاسم:

25/8/2025

التاريخ:

Dynamic Contrast Enhanced MRI Study of the Breast :

Pulse Sequences :

Pre-contrast axial T1, T2, STIR and Sagittal T2WIs.

Post-contrast axial VIBE and Subtraction WIs.

Indication of the examination:

38 years old female patient complaining of right breast lump.

Report :

- Both breasts show mixed fibro glandular parenchyma (ACR B) with mild background enhancement.
- Right breast shows Multicentric breast lesions seen at 12 to 5 o'clock exhibiting intermediate T1 , Low T2 , bright STIR signal showing heterogeneous and rim contrast enhancement the largest lesion opposite 3-4 o'clock measures 2.7 x2.6 cm.
- Normal left breast.
- Normal nipple areola complex of both breasts.
- Bilateral non-specific axillary L.Ns with normal cortical thickness and preserved fatty hilum.

OPINION:

- Known case of histopathologically proven right breast cancer showing Multicentric breast lesions as described (BIRADS VI).
- Normal left breast MRI examination (BIRADS I).



DR. HEBA NABIL, M.D

Women imaging Consultant

Patient Name: Heba Ismael Abdel Gawad

Date: 11/10/2025

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Referring physician: Prof.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No definite active osseous metastatic bony lesions
- Spondylodegenerative changes are seen at D/L spine
- Bilateral shoulders, both knees, joints are seen with osteoarthritic changes, .
- Both kidneys are normally seen.

Conclusion:

- No evidence of active osseous bony lesions could be seen all over the surveyed body,

Nuclear Medicine Unit

DR. Sherien Mahdy, MCD

DR/ Mohamed Emam